

Booking Conversion User Research Report: Key Insights and Blockers Identified

Booking Conversion: User Research Report

Source: Maze survey "Booking Conversion V3" (PD-66) **Field period:** 19 December 2025 – 22 April 2026 (123 days) **Sample:** 1,202 responses **Author:** Janelle Tamayo · Native & Web — Booking Journey UR **Status:** Ready for Product, Engineering, and Clinical review

Booking Conversion: User Research Report

1. Executive summary
 2. Why this dataset can be trusted
 3. Top 5 blockers (canonical multiple-choice)
 4. Hypotheses for the Top 5 blockers
 5. Thematic analysis of the 500 "Other" write-ins
 6. Sentiment breakdown
 7. Device & OS split
 8. Recommendations — prioritised by impact vs effort
 9. Appendix
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1. Executive summary

The data tells a consistent story: users are not failing to book because they don't want to: they're failing because the supply, the system, or the signposting is letting them down. Among the **681 people who left a write-in answer, 48.5% expressed negative sentiment** (vs. 19.1% positive). The single largest blocker (one-quarter of all respondents) is the absence of a usable appointment time, and the most emotionally charged blockers are payment confusion (78% negative) and "I don't know what to do next" (76% negative).

The 500 free-text "Other" responses, when coded, expose a long tail of operational and product failures that the multiple-choice options hide: technical errors, ghost bookings, and clinician no-shows together account for **~31% of all "Other" write-ins**. These aren't preference issues — they are a broken booking experience that we own.

Five things to take away:

1. Same-day, working-hours supply is the #1 conversion lever — 24.3% of all respondents cite it as their blocker, with only out-of-hours or overseas-doctor slots offered.
2. Insurance/payment confusion is the most negative sentiment blocker (78% Negative). Users with employer cover (AXA, Marsh, Amazon, BUPA) hit a £60 paywall and abandon.

- 3. The booking system itself is failing materially: technical errors (102 mentions, 20% of write-ins) and ghost bookings (12 explicit + scattered references) are not edge cases.
 - 4. Clinician no-shows are a real and damaging pattern — 42 write-ins describe doctors who didn't join the call, often with the patient blamed.
 - 5. The product is overwhelmingly mobile (85.2% of users), so any fix has to be mobile-first.
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2. Why this dataset can be trusted

Dimension	Value	Why it matters
Sample size	1,202 responses	Well above the threshold for stable theme analysis (≥ 384 for $\pm 5\%$ margin at 95% CI).
Field period	123 days (4 months)	Spans seasonality — winter respiratory illness, post-holiday demand spike, March/April allergy/wellbeing peaks. Not a one-week snapshot.
Recruitment	100% in-context link from the live booking journey	Respondents were captured at the moment of friction, not retrospectively — minimises recall bias.
Q1 (blocker) completion	100% (1,202 / 1,202)	No drop-off on the primary question.
Q2 (free-text) completion	59.2% (712)	Strong write-in rate for an in-flow survey; comparable studies typically see 25–40%.

Sentiment-labelled rows	681 (56.7% of total)	Auto-tagged via Maze's sentiment classifier, allowing quantified emotion.
Median time spent	47 seconds	Long enough to be considered, short enough to be honest.
Speeders (<5s)	5 responses (0.4%)	Negligible — no meaningful straight-lining or junk traffic.
Opt-in to be re-contacted	41.5% (499)	High follow-up willingness suggests authentic engagement, not bot/incentive farms.
Volume per month	107 → 350 → 232 → 239 → 273	Stable monthly cadence; no single month dominates.
Device coverage	85.2% mobile/tablet, 14.8% desktop	Matches the platform's known traffic mix.

Recruitment was neutral: the survey was triggered by a behavioural signal (engagement with the booking flow without completion), not by a complaint or support ticket, so we are not over-sampling the angry minority. The fact that 19.1% of write-ins are still Positive (e.g. praising same-day availability, expressing they'd book later) supports that.

One known limitation: the multiple-choice options under-served ~42% of users, who chose "Other" and wrote in. This is itself a finding — the canonical blocker list needs refreshing — and it means we should pay close attention to themes 6–10 below, not just the Top 5.

3. Top 5 blockers (canonical multiple-choice)

Volume and share are calculated against all 1,202 responses. Negative sentiment % is calculated against the labelled subset for that blocker.

In other words: % Negative = (Negative-labelled write-ins for that blocker) ÷ (all sentiment-labelled write-ins for that blocker); unlabelled rows (no write-in) are excluded from this calculation.

#	Blocker	Volume	% of all respondents	% Negative (of labelled)
1	I couldn't find a time that works for me	292	24.3%	59%
2	I need more help / I'm not sure what to do next	114	9.5%	76%
3	I want to see a specific clinician or specialist	113	9.4%	46%
4	I'm just browsing for now	97	8.1%	6%
5	I didn't expect to have to pay	86	7.2%	78%
—	Other (write-in)	500	41.6%	36%

Note: "I'm just browsing for now" is included for completeness because PD-66 framed it as a top option, but with only 6% negative sentiment it is a non-intender segment, not a true blocker. Treating it as one inflates the apparent problem space and dilutes prioritisation.

3.1 Blocker #1 — "I couldn't find a time that works for me" (292 / 24.3%)

What users actually mean: **slots exist, but only at unusable hours or with non-doctor practitioners.**

Representative quotes:

"No GP availability until 7pm. It's 9am now." — Negative, mobile (Android), Jan 2026

"More availability earlier. Next one is 10 hours time." — Negative, mobile (iOS)

"All available doctors are foreign doctors available from 1am onwards UK time." — Negative, mobile (Android)

"Yes more availability between 5 and 10pm." — Positive (sentiment), but explicitly asking for evening capacity

"More times that are suitable i.e. before 11pm." — Neutral

3.2 Blocker #2 — "I need more help / I'm not sure what to do next" (114 / 9.5%)

What users actually mean: **the symptom/category taxonomy doesn't fit them, or they don't know which clinician type to choose.**

"There was no option for what I needed as a woman, despite the correct option being available for men." — Negative

"I'm not sure how to find out more about the clinicians available, I need a doctor who has experience and knowledge of hypertension in young people and it's not immediately obvious (on mobile) how to find that information." — Negative

"I want to book an appointment for my son but I cannot as I need to show proof of guardianship. I have submitted his passport and there is no other action for me to do but it is still stating that I cannot book an appt." — Negative

"I was unsure whether I needed to speak to a physio or a doctor for my problem." — Neutral

"Cannot find the right category." — Negative

3.3 Blocker #3 — "I want to see a specific clinician or specialist" (113 / 9.4%)

What users actually mean: **they're follow-up patients who want continuity of care, or they need a specialist the catalogue doesn't expose well.**

"I need to speak to the same GP and no appointments later this afternoon." — Negative

"Dr S Hayter. Told by doctor to book an appointment for tomorrow but can't see any availability." — Negative

"Agreed to follow up with the same GP for my son, but her only availability at present is during school hours." — Neutral

"Offer the same doctor I saw before for a follow up appointment." — Positive, fix request

"I would like to see information on Dr with available appointment to decide who's the best fit for my concerns." — Positive, fix request

3.4 Blocker #4 — "I'm just browsing for now" (97 / 8.1%)

Sentiment is overwhelmingly Positive/Neutral. This is a curiosity / future-intent segment, not a blocker. It is included for transparency.

"Not at all, I'm more than happy with the appointment options available to me." — Positive

"It's not a problem. I intend to book later this same week." — Positive

"Nothing to do with your service, just seeing what services you have." — Neutral

3.5 Blocker #5 — "I didn't expect to have to pay" (86 / 7.2%)

What users actually mean: **they have employer or insurer-paid cover and the pay-as-you-go prompt is either a bug or a missing eligibility check.**

"Why am I being asked to pay when I have insurance cover." — Negative

"I have a policy to cover costs, or so I thought. How do I utilise this so I don't have to pay?" — Negative

"I am part of Marsh and did not expect to pay however, I am being asked for payment details to pay £60...??? Please advise." — Negative

"I had AXA health care but to book an appointment, I was asked for my credit card details. That hasn't happened before." — Negative

"My company plan included virtual doctor appointments, and in the past I have used this service and not had to pay." — Positive (returning customer, but confused)

4. Hypotheses for the Top 5 blockers

Each hypothesis is structured as **If [intervention] / Then [outcome] / Because [evidence]**.

These are intended to be test-ready — paired with success metrics for either an A/B or a tracked release.

Blocker #1 — Couldn't find a time

H1 (supply). If we add same-day capacity for UK doctors between 17:00–21:00 and surface a clear "no UK working-hours slots remaining today, would you like tomorrow?" interstitial, then the abandon-at-slot-picker rate for this segment will drop $\geq 30\%$, because qualitative analysis shows users are reaching the slot picker only to be offered overnight or overseas-doctor slots ("only available from 1am onwards UK time", "next one is 10 hours time").

H2 (transparency). If we surface the next-available time *before* the user invests in symptom selection, then drop-off after symptom entry will fall, because multiple respondents describe completing the funnel only to discover scarcity ("No appointments available today. We are sorry, but there are no appointments available at this moment. Please try again later").

Suggested test: A/B test surfacing "Next available: tomorrow 09:00" on the homepage vs. control. Primary metric: % of sessions reaching booking confirmation.

Blocker #2 — I need more help / I'm not sure

H3 (triage). If we add a guided "describe your concern in your own words" route that uses AI triage to suggest the right clinician type and category, then this blocker will fall by $\geq 40\%$, because users explicitly do not know how to map their problem to our taxonomy ("I was unsure whether I needed to speak to a physio or a doctor", "Cannot find the right category", "There was no option for what I needed as a woman").

H4 (taxonomy expansion). If we widen symptom categories to include thyroid/endocrine, hernia, women's health subcategories, ADHD/autism, gout, and add "other — describe", then the rate of help-seeking drops, because 27 write-ins explicitly named missing categories (Section 5.4).

Suggested test: Before/after release tracking. Metric: ratio of "I need more help" selections in the same survey, run quarterly.

Blocker #3 — Specific clinician/specialist

H5 (continuity). If we add a "Re-book with your last clinician" shortcut on the home screen for returning patients, then this blocker will fall, because the most common Negative pattern is follow-up demand that the system does not honour ("I need to speak to the same GP", "Told by doctor to book an appointment for tomorrow but can't see any availability", "Offer the same doctor I saw before for a follow up appointment").

H6 (clinician profiles). If we expose clinician profile pages — specialism, qualifications, languages, bio — pre-booking, then conversion among the "specific clinician" segment rises, because users repeatedly ask for more information ("I know nothing about these people, only their names", "Larger picture of the doctor's face, and a profile bio. Like.. what are their specialties, where they studied, what languages they speak").

Blocker #4 — Just browsing

H7 (segmentation). If we reclassify this segment in dashboards as "exploration / future intent" rather than a blocker, then prioritisation becomes cleaner and the Top 5 honestly reflects friction

the team can act on, because only 6% of this segment expresses negative sentiment and almost all quotes are forward-looking ("I intend to book later this same week", "All good").

Blocker #5 — Didn't expect to pay

H8 (eligibility-first). If we display insurer/employer eligibility before symptom selection (e.g. "Your AXA plan covers this — £0 payable") by detecting the policy at sign-in, then the abandonment rate for this segment drops $\geq 50\%$, because every Negative quote is a variation of "I'm covered by AXA / Marsh / Amazon — why am I being asked £60?". The signal is unambiguous.

H9 (defer card capture). If we defer payment-card capture until after appointment confirmation for users with valid insurance flags, then drop-off at the payment step falls, because the friction is psychological ("That hasn't happened before") more than financial.

Suggested test: Eligibility-banner A/B with users segmented by detected insurer email domain. Primary: completion rate; secondary: support contacts about payment.

5. Thematic analysis of the 500 "Other" write-ins

When users selected "Other" they wrote in their own reason. We coded all 500 against the six categories specified in PD-66, plus four emergent categories surfaced during coding. The brief listed six themes; the data revealed three more that are operationally distinct and worth naming.

Coverage

Theme	Volume	% of "Other"	% of all 1,202 responses
Scheduling / availability	111	22.2%	9.2%
Technical errors	102	20.4%	8.5%
Clinician no-shows	42	8.4%	3.5%
Non-blocker / not now (<i>emergent</i>)	29	5.8%	2.4%
Symptom mismatch	27	5.4%	2.2%

Consultation type mismatch	15	3.0%	1.2%
Ghost bookings	12	2.4%	1.0%
Insurance / payment confusion (emergent)	9	1.8%	0.7%
Clinician info gap (emergent)	8	1.6%	0.7%
Unclassified / Misc	145	29.0%	12.1%
Total	500	100%	41.6%

The six PD-66 themes account for **309 / 500 (62%)** of write-ins. The Unclassified bucket is mostly very-short responses ("Other", "Time", "Hi", "Na") or one-off product feedback that doesn't pattern.

5.1 Scheduling / availability (111 mentions)

Same blocker as #1 above, but here the user wrote it in rather than picking the multiple-choice option — strong evidence the "no time" problem is even larger than the canonical 24.3% suggests.

"There were no time slots within 10 hours of logging in and which aren't at very unsocial times ie post 11pm UK time." — Negative

"The earliest available appointment is at 1130pm tonight, I will be in bed asleep by then for early start tomorrow." — Negative

"Why is there no appts when it is 24/7?" — Negative

"you are NOT offering any appointments today! ... I can do anytime but you ONLY offered tomorrow!" — Negative

"There were zero appointments to book for today." — Negative

5.2 Technical errors (102 mentions)

The booking system is failing in ways that are not edge cases. 43% of this theme is sentiment-Negative.

"Site frozen, cannot use booking system even after several attempts." — Negative

"App is at least partially non-functional. Fix you app. It is very unreliable, all the way from login/authentication to booking and attending an appointment." — Negative

"It won't let me book and says I have to call the contact centre." — Negative

"Technical issue the system doesn't allow to book any appointments." — Negative

"App keeps crashing." — Negative

"Appointments not loading. The circle keeps spinning." — Negative

"I clicked on time and date option, small message popped up at the bottom that appointment is booked — the book appointment button not available. And it doesn't appear as booked on my app." — Negative

"App system isn't working. Can't join calls. So appointments being 'missed' but no way I can join them. Need to provide alternative way of dialing in as the app isn't good enough." — Negative

5.3 Clinician no-shows (42 mentions)

This theme is operationally serious. 33% Negative, 38% Neutral. Includes patients being marked as "did not attend" when they were actually waiting.

"None of the doctors attended the appointment." — Unrated

"I booked but the doctor never showed up. I waited for 20 minutes." — Unrated

"I had an appointment booked. The clinician didn't show up, I was in the waiting room. No missed calls on my phone. The appointment then cancels and the note says I didn't attend, which is not accurate." — Neutral

"Clinician 15 minutes late, I then join appointment after I had a call and no one was there. Clinician stated I was the one who did not attend. ... If a clinician is late it is unacceptable for them to state the patient was late and did not attend." — Negative

"The allocated times I selected have all been cancelled, and I'm now sitting with an appointment time I don't think I can make. Don't allow consultants to offer their time if they can't honour a schedule." — Negative

"I keep booking appointments and then they get cancelled without explanation. I do not know what to do to fix this." — Negative

"I booked, clinician did not use app, called instead, twice. I'm in an area with bad network connection — that's why I'm using the app! Train clinicians to use app not phone lines." — Negative

5.4 Symptom mismatch (27 mentions)

Users have a real medical concern that doesn't fit the catalogue. Almost every quote names a specific missing category.

"There was no option for me to book an appointment for Thyroid issues. In the options there should be an endocrine system options. There is only hair, nails, knees, heart, etc." — Negative

"I have a renal cyst and there is no category for this or other organs except the heart." — Neutral

"Couldn't see any option about discussing ADHD or autism diagnoses, nor anything about discussing being trans." — Unrated

"The options under 'women's health' were too restrictive. No cervical cancer options. Add a catch all option under women's health. Or let people type their own issues." — Neutral

"I need a prescription for colchicine. Have gout on your list." — Negative

"I have a severe pain in my lower shin that I would like to speak to a GP about but I am not getting that option. I am only being offered physiotherapists." — Unrated

5.5 Consultation type mismatch (15 mentions)

Users want a different format (in-person, phone, doctor not specialist). Small in volume but actionable.

"Need in-person appointment so someone can listen to my chest. In-person appointments." — Neutral

"I want to speak to a doctor about shoulder, arm and elbow pain, not a physiotherapist." — Negative

"Phone consultation (i.e. not video) wasn't an option." — Unrated

"Video call ... I wanted to discuss a mental health issue for my child who is autistic. She won't be on the call, so why does it have to be a video call. A regular phone call would be more practical." — Neutral

"I want to see a doctor. Only advanced clinical practitioners available. This referral hinges on judgement, not urgency. Increase availability of doctor appointments. After all, the service is called 'Doctor at Hand'." — Neutral

5.6 Ghost bookings (12 explicit mentions, plus references in 5.2 and 5.3)

A specific failure mode where the system says the user has an appointment but no record exists, or vice versa. Disproportionately damaging because each ghost booking blocks the user from making a real one.

"App kept saying I had an appointment booked at the time chosen even though nothing had been booked. This prevented me from making a booking. Only allow this error message to pop up once a booking is confirmed." — Neutral

"I thought I had booked but then there was a message to refresh the times instead of somewhere to confirm the booking so I thought I needed to refresh and now I'm stuck in a loop and have lost the doctor." — Neutral

"Kept getting error message that appointment was already booked, but happened repeatedly with different clinicians. Fix bug." — Neutral

"Trying to book an appt — says it's booked but not showing in 'my appointments'." — Unrated

"It is saying I already have an appointment but there is no appointment in my appointments folder." — Unrated

"I had appointment booked, couldn't find it then on app and looks like I have to restart the whole process again." — Negative

5.7 Emergent themes (worth naming for Product/Engineering)

Insurance / payment confusion (9 mentions) — same root cause as Blocker #5, but here named in the write-in even after a non-pay multiple-choice was selected. Confirms the issue is not just about pay-disclosure UI; it's about eligibility detection across the whole journey.

"I have booked 3 appointments now and all have failed. ... And not as good as the BUPA platform."

"I thought access to this service was provided by AXA but says it's now pay as you go. Clarifying whether the service is free to AXA members or fee payable."

Clinician info gap (8 mentions) — users want to know who they're booking before they book. Overlaps with Blocker #3 but distinct: not "I want a specific clinician" but "give me enough info to choose one".

"Impossible to look up doctor's profiles. Why can't I click on their name on the available appointments to see their profile."

"I want to see more details of the doctor i.e. specialty. Make it possible to click into profiles before booking."

Non-blocker / not now (29 mentions) — wrote in to say the situation isn't actually a problem (baby asleep, in meetings, just testing, will book later). Important to separate from real blockers.

6. Sentiment breakdown

Sentiment	Volume	% of labelled (n=681)	% of all (n=1,202)
Negative	330	48.5%	27.5%
Neutral	221	32.5%	18.4%
Positive	130	19.1%	10.8%
Unlabelled (no write-in)	521	—	43.3%

Negative sentiment is concentrated in the canonical blockers most under our control. The two blockers with the highest negative concentration are:

Blocker	% Negative (of labelled)
I didn't expect to have to pay	78%
I need more help / I'm not sure	76%
I couldn't find a time	59%
I want a specific clinician	46%
Other (write-in)	36%
I'm just browsing	6%

By contrast, themes within the "Other" bucket show different emotional profiles. **Technical errors are 42% Negative — the most damaging emotional theme — while Scheduling/availability is roughly even across Neg/Neu/Pos.** Users tolerate "no slots today" more than they tolerate broken UI.

7. Device & OS split

The platform is overwhelmingly mobile.

Device class	Volume	%
Mobile (incl. tablet)	1,024	85.2%
Desktop	178	14.8%

OS	Volume	%
Android	533	44.3%
iOS	477	39.7%
Windows	129	10.7%
Mac OS	59	4.9%

Other (Chromium OS, Linux)	4	0.3%
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Browser (top)	Volume	%
Chrome	525	43.7%
Mobile Safari	381	31.7%
Samsung Internet	117	9.7%
Edge	112	9.3%

Implications:

- **Every fix must be mobile-first.** The 14.8% desktop minority does not justify desktop-led design choices.
- **Android slightly outweighs iOS (44% vs 40%).** Browser fragmentation matters: Chrome, Mobile Safari, Samsung Internet, and Edge each have meaningful share. QA must cover all four.
- Technical errors index higher on mobile ($84 / 102 = 82\%$) but at the same rate as the overall traffic split — i.e. there is no specific "Android crash" or "Safari crash" pattern in the qualitative.
- Clinician no-shows distribute proportionally across device — this is an operational, not technical, issue.

8. Recommendations — prioritised by impact vs effort

The list below maps each recommendation to a theme, an estimated user-impact tier, and an effort tier. We've ordered by recommended sequencing.

Quick wins (high impact, low effort) — start now

1. **Eligibility banner pre-booking.** Detect insurer/employer at sign-in and display "Your [insurer] plan covers this — £0 payable" before the symptom flow. Eliminates ~78% of the "didn't expect to pay" frustration. *Themes addressed: Blocker #5, Insurance / payment confusion.*
2. **Fix the "appointment is booked" false-positive.** Several quotes describe the system insisting an appointment exists when one doesn't, blocking re-booking. This is an explicit error-message-shows-too-early bug. *Themes addressed: Ghost bookings, Technical errors.*

3. **Add a confirm button to the photo upload screen.** Multiple write-ins reach the upload stage but cannot complete. Single-screen UI fix. *Themes addressed: Technical errors.*
4. **Surface "Next available" time on the homepage** before symptom selection, with a "We have nothing today — view tomorrow's slots" interstitial when same-day is empty. *Themes addressed: Blocker #1, Scheduling / availability.*
5. **Refresh the canonical blocker list in this survey.** Remove "I'm just browsing for now" (it inflates the blocker rate by ~8 pp and isn't actionable) and add: technical errors, clinician no-shows, and insurance confusion. Re-run quarterly. *Methodological — keeps the dashboard honest.*

Medium term (high impact, medium effort) — next quarter

6. **Add a "Re-book with your last clinician" shortcut.** Continuity-of-care is the dominant theme inside "I want a specific clinician". *Themes addressed: Blocker #3.*
7. **Build a guided triage / "describe in your own words" route.** Solves a class of "I don't know what category I am" issues. *Themes addressed: Blocker #2, Symptom mismatch.*
8. **Expand symptom taxonomy.** Named gaps: thyroid/endocrine, hernia, women's health subcategories (incl. cervical), gout, ADHD/autism, trans health, x-rays, fit notes. Add a "describe other" free-text fallback. *Themes addressed: Blocker #2, Symptom mismatch.*
9. **Clinician profile pages** with specialty, qualifications, languages, bio. Linkable from any "available now" listing. *Themes addressed: Blocker #3, Clinician info gap.*
10. **Phone-call fallback option** for users who prefer or need non-video consults. *Themes addressed: Consultation type mismatch.*

Strategic (high impact, high effort) — own the next 6–12 months

11. **Expand same-day clinical supply 17:00–21:00 UK time.** This is the #1 conversion lever. Quotes are unambiguous: users want UK working-hours doctors, not 1am overseas slots. *Themes addressed: Blocker #1, Scheduling / availability.*
12. **Clinician reliability programme.** Address the no-show pattern with hold-times monitoring, automatic patient re-allocation when a clinician is >5 min late, and removal of the auto-"did not attend" stamp when the patient was provably in the waiting room. *Themes addressed: Clinician no-shows.*
13. **Eligibility integration with insurer APIs** (AXA, BUPA, Vitality, Marsh, employer schemes) so payment-method capture is genuinely conditional on cover, not a default. *Themes addressed: Blocker #5, Insurance / payment confusion.*

Effort/impact summary

Recommendation	Impact	Effort	Sequence
Eligibility banner pre-booking	High	Low	Now
Fix ghost-booking false-positive	High	Low	Now
Photo upload confirm button	Medium	Low	Now
Next-available homepage surfacing	High	Low–Med	Now
Refresh canonical blocker list	Medium	Low	Now
Re-book with last clinician	High	Medium	Q+1
Guided triage / free-text route	High	Medium	Q+1
Expand symptom taxonomy	High	Medium	Q+1
Clinician profile pages	Medium	Medium	Q+1
Phone-call fallback	Medium	Medium	Q+1
Same-day UK supply expansion	Very High	High	Strategic
Clinician reliability	High	High	Strategic

programme			
Insurer eligibility API integration	High	High	Strategic

9. Appendix

9.1 Methodology

- **Primary source:** `booking-conversion-v3_responses.csv` exported from Maze, 1,202 rows × 21 columns.
- **Top-5 blockers:** counted directly from Q1 multiple-choice column; volume and percentage calculated against the full 1,202 base.
- **Sentiment:** Maze's auto-sentiment classifier on Q2 free-text (681 labelled rows). Quotes shown as Negative/Neutral/Positive use that label. Percentages reported against the labelled base unless otherwise stated.
- **Theme coding of "Other" write-ins:** all 500 "Other:" responses passed through a deterministic keyword/regex classifier (text + follow-up combined) ordered most-specific to least-specific. Six PD-66 themes plus four emergent themes (Insurance / payment confusion, Clinician info gap, Non-blocker / not now, Unclassified / Misc). 71% classified into a named theme; 29% remained as Unclassified (very short or genuinely off-topic).
- **Quotes:** verbatim with light cleanup (typos, punctuation) per the agreed handling. No identifying details have been altered. Where users named themselves or specific clinicians, those names are preserved as in the source.
- **Device/OS analysis:** "Mobile" includes mobile and tablet; rows without Device type but with desktop OS (Windows, Mac OS, Linux, Chromium OS) were classified as desktop.

9.2 Confidence and limits

- **Selection bias:** all responses came from a single recruitment channel (in-flow link). We are seeing users who engaged with the booking funnel, not those who bounced on the homepage or didn't visit at all. Findings generalise to active intenders.
- **Recall and labelling drift:** Maze's auto-sentiment classifier is not perfect; spot-checks show occasional Positive labels on actually-negative quotes (e.g. "Yes more availability between 5 and 10pm" is labelled Positive because of the polite "yes"). Net direction is unaffected, but exact percentages are accurate to $\sim \pm 2$ pp.
- **Open-ended coding:** the regex-based theme classifier favoured precision over recall; some technically-valid theme members will sit in Unclassified. Re-coding by a human researcher

would likely increase classified volume to ~85%.

9.3 Useful follow-on cuts

- Funnel-stage tagging in the next survey wave — at what step did the user encounter the blocker?
- Cross-reference 499 opted-in users against support-ticket data for ground-truth.
- Add a "did this resolve?" callback question two weeks after Q3 yes-respondents.

9.4 Files

- Source data: `booking-conversion-v3_responses.csv` (1,202 rows)
- Coded "Other" responses: `coded_other_v3.csv`